

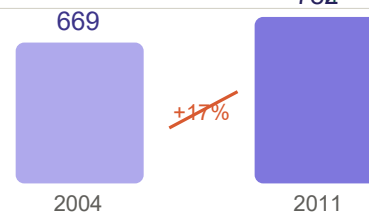
Inequalities in antenatal care use: a comparative analysis

CONTEXT

Maternal mortality ratio rose from 669 (2004) to 782 (2011) — a +16.9% increase.

DHS surveys highlighted persistent care disparities.

MATERNAL MORTALITY RATIO per 100,000



OBJECTIVE

Understand the dynamics of inequalities in demand for antenatal care and their determinants across two reference periods.

STUDY DESIGN & METHODS

TARGET POPULATION

Women aged 15–49 with ≥1 live birth in the 5 preceding years

DATA SOURCES

DHS Cameroon
2004 · 2011
Two-stage area sampling

CONCENTRATION CURVE

Measures socio-economic inequality in ANC use

MULTI-COMPONENT ANALYSIS

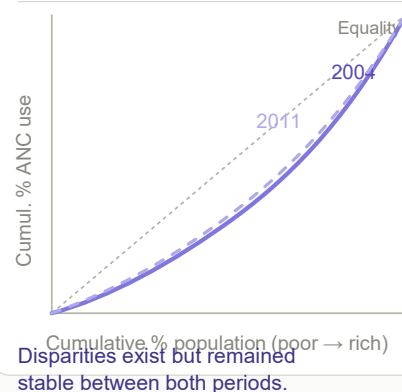
Decomposes inequality drivers

MULTILEVEL ANALYSIS

Controls for individual & contextual factors

KEY FINDINGS

CONCENTRATION CURVES



FINDING 1 · LEVEL OF INEQUALITY

2004

Disparities present

≈
No change

2011

Disparities present

FINDING 2 · DETERMINANTS OF INEQUALITY (both periods)

Region of residence

Woman's education

Husband's education

Household well-being

Sex of household head

FINDING 3 · STRUCTURAL STABILITY, OPERATIONAL CHANGES

Structure of factors unchanged between 2004 and 2011, but how each factor operates differs across the two periods.

Same determinants

Different pathways

CONCLUSION & IMPLICATIONS

ANC inequalities persisted from 2004 to 2011. Reducing them requires targeted actions on education, wealth, and regional factors.

Systematising ANC consultations is key to reducing maternal mortality.

METHODS LEGEND

■ 2004 DHS data ■ 2011 DHS data ■ Determinant / finding ■ Nuanced finding

DHS = Demographic and Health Survey · ANC = Antenatal care · Methods: concentration curve, multi-component analysis, multilevel analysis

Population: women aged 15–49 with ≥1 live birth in 5 years preceding each survey

Visual abstract · ResearchGate 2022